RURAL HEALTH TRANSFORMATION · 50-STATE SNAPSHOT

Rural Health Transformation by the numbers

States proposed the care backbone and digital rails. The next test is connecting investment to care delivery.

By Martin Blanco · github.com/mbvibecode · Prepared October 2026.

FUNDING · REPORTED THROUGH JULY 31, 2026

Nearly $10B awarded; $4.001B reported obligated.

$9.995B	$4.001B	40.0%	48 / 50
PERIOD 1 AWARDS	REPORTED OBLIGATED	OF AWARD TOTAL	STATES REPORTING > $0

Obligations are state-reported commitments—not expenditures, outcomes, or a complete view of progress.

WHAT STATES PROPOSED · DECEMBER 2025 ABSTRACTS

Care backbone, then digital rails.

CARE BACKBONE

50	47	47	44/50
WORKFORCE	PREVENTION / CHRONIC CARE	NETWORKS / COORDINATION	NAME ALL THREE

DIGITAL RAILS

46	43	40	37/50
ANY TECHNOLOGY CATEGORY	DATA / RECORDS	TELEHEALTH / REMOTE CARE	NAME DATA + TELEHEALTH

Counts show mentions among 50 states; categories overlap.

THE CARE CONNECTION

Technology is common. Explicit linkage is not.

10 of 34
BEHAVIORAL-HEALTH STATES
explicitly connect technology

3 of 11
HOME / LTC STATES
explicitly connect technology

Every state naming either care area also mentions technology somewhere; these counts require an explicit care-area link.

FROM MONEY TO CARE

Follow dollars into the workflow.

Track procurement, active use, reach, referral closure, continuity, and outcomes—not purchases alone.

COMMIT **Obligation** → ACTIVATE **Workflow use** → MEASURE **Care results**

METHODS & READING NOTES

PROPOSAL METRICS	Analyst-curated coding of 50 public state abstracts from December 2025; categories overlap.
FUNDING METRICS	State-submitted Year 1 reporting through July 31, 2026; $9.995B award denominator.
INTERPRETATION	Mentions do not establish implementation; obligations are commitments, not expenditures/outlays or outcomes.

Explore the state evidence · View the obligations source · RHT INFOGRAPHIC